



Notifications in Mental Health

Policy code	SMHS:165
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Date of current endorsement	02 February 2024
Functional subgroup	Mental Health
Summary	The Mental Health Act 2014 provides that certain events affecting persons under the Act require mental health services to notify the person's personal support person and/or an external person or body such as the Chief Psychiatrist. Other events require mental health services, by way of legislation or policy, to notify an external person or body. Staff within SMHS will ensure they comply with all notifications requirements within the Mental Health Act 2014 and within this policy.
Policy owner	Executive Director Clinical Service Planning and Population Health, Kate Gatti
Contact	Health Service Planner, Clinical Service Planning, Anureet Johl, 6152 1169
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NSHQHS Standards	Standard 1: Clinical Governance Standard 2: Partnering with Consumers
Chief Psychiatrists Standards for Clinical Care	Consumer and carer involvement in individual care
Status	Active

1. Background

1.1. About this document

The policy is designed to guide staff regarding the process for notification of certain events. The Mental Health Act 2014 (MHA 2014) lays an obligation on Mental Health Services to notify a carer, close family member or personal support person (PSP) when these events occur.

Notification of certain events will be made as required to the following: Carers, close family members and/or other support person,

- Chief Psychiatrist (OCP),
- Chief Mental Health Advocate (MHAS),
- Mental Health Tribunal (MHT), and
- Mentally Impaired Accused Review Board (MIARB).

Refer to [Appendix A Notification Matrix](#)

1.2. Key definitions

Term	Definition
Person responsible	Schedule 2 of the MHA 2014 states the person responsible for notification of a particular event. Tables in the appendix of this policy identify the person responsible for each notification event
Personal support person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, nominated person, the parent or guardian of a child and any guardian or enduring guardian of an adult (MHA 2014 s.7.

2. Policy Statement

The Mental Health Act 2014 (MHA 2014) provides that certain events affecting persons under the Act require mental health services to notify the person's personal support person and/or an external person or body such as the Chief Psychiatrist.

Other events require mental health services, by way of legislation or policy, to notify an external person or body.

Staff within SMHS will ensure they comply with all notifications requirements within the Mental Health Act 2014 and within this policy.

3. Roles and Responsibilities

3.1. Executive

Each SMHS site Service Director or Co-Director will ensure that managers and supervisors are aware of the requirement to implement guidelines and safe work procedures relating to MHA 2014.

3.2. Managers/Supervisors

Managers and supervisors should:

- Ensure the development of relevant protocols and procedures
- Where available and appropriate, ensure appropriate training and education is provided to staff in relation to application of MHA 2014.

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3.3. Staff

Staff should:

- Comply with mandatory training requirements and proactively seek further training as required
- Ask for assistance if required and involve relevant medical staff in the decision-making process regarding MHA 2014.

4. Mental Health Tribunal approvals

Certain treatments require approval by the Mental Health Tribunal before they may lawfully be performed. These treatments are:

- Electroconvulsive therapy on a child over the age of 14 who is a voluntary patient;
- Electroconvulsive therapy on a child over the age of 14 who is an involuntary patient;
- Electroconvulsive therapy on an adult involuntary patient;
- Psychosurgery on an adult or on a child over the age of 16.
- Refer to [Mental Health Act 2014](#) s. 195.
- Refer to [Mental Health Act 2014](#) s. 196.
- Refer to [Mental Health Act 2014](#) s. 198.
- Refer to [Mental Health Act 2014](#) s. 208.

5. How carer, close family member or other personal support person is to be notified of notifiable event

Where a notifiable event occurs, the person responsible must ensure that any carer, close family member or other personal support person (hereafter “carer.”) of a person is notified of the notifiable event as soon as practicable after it occurs.

The requirement to notify is taken to have been complied with if the person responsible makes reasonable efforts to notify the carer until either:

- At least one carer is notified; or
- It is reasonable for the person responsible to conclude that it is not possible for any carer to be notified.
- Refer to [Mental Health Act 2014](#) s. 140.
- Refer to [Mental Health Act 2014](#) s. 141.

6. Best interests of a person

A carer etc. is not entitled to be notified that any other notifiable event has occurred if a psychiatrist believes it is not in the best interests of the person for the carer, close family member or other personal support person to be notified of the event.

In deciding what is in a person’s best interests, a medical practitioner, authorised mental health practitioner or psychiatrist must have regard to:

- the person’s wishes (to the extent that it is practicable to ascertain those wishes).
- the views of the person’s ensuring guardian or guardian, parent or guardian, nominated person, carer and close family member, as applicable; and
- any other matter that the person or body considers relevant to making the decision.

Any decision that a carer etc. is not entitled to be notified must be recorded in accordance with the MHA 2014 and documented in the patient’s DMR with a clear rationale.

If the carer requests to be notified of the event, the decision-maker must:

- advise the carer of the decision to not advise, and the reasons for it; and file a record of the advice and give a copy to the patient.

The carer may request a written confirmation regarding advice provided. The decision-maker must comply with the request, file a copy of the written confirmation and give a copy of the written confirmation to the patient.

The decision to not notify a carer etc. may be revoked. If so, the decision-maker must file a record of the revocation and the reasons for it.

If the carer etc. previously requested to be notified of the event, the decision-maker must ensure that as soon as practicable:

- they are notified of the event; and
- a record of when and how they were notified is filed and a copy given to the patient.
- Refer to [Mental Health Act 2014](#) s. 142(1).
- Refer to [Mental Health Act 2014](#) s. 142(2).
- Refer to [Mental Health Act 2014](#) s. 7.

7. Record keeping

Staff must ensure they document notifications in accordance with their obligations under the Mental Health Act 2014 in PSOLIS.

8. Compliance monitoring

8.1. Compliance monitoring

Each SMHS site Service Director or Co-Director is to ensure compliance with this policy.

8.2. Legislative obligations

A staff member of a mental health service who reasonably suspects that a reportable incident as defined by s. 254 of the MHA 2014 has occurred must report the suspicion to the person in charge of the mental health service or the Chief Psychiatrist.

Penalty: a fine of \$6000.

A person in charge of a mental health service who becomes aware of the occurrence of a notifiable incident as defined by s. 525 of the MHA 2014 must, as soon as practicable, report the occurrence to the Chief Psychiatrist.

Penalty: a fine of \$6000.

- Refer to [Mental Health Act 2014](#) s. 254(2).
- Refer to [Mental Health Act 2014](#) s. 526(2).

9. Related Documents

9.1. Legislation

- [Mental Health Act 2014](#)
- [Mental Health Regulations 2015](#)

9.2. Additional references

- Refer to Office of the Chief Psychiatrist [Reporting Notifiable Incidents - Public Mental Health Services](#)
- Refer to Office of the Chief Psychiatrist [Monitoring and Reporting](#)
- Refer to Office of the Chief Psychiatrist [Chief Psychiatrist's Standards for Clinical Care](#)

9.3. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP 4	05/02/2024	02/02/2024	02/02/2027	2027	

9.4. Approval

Approval by	Executive Director Clinical Service Planning and Population Health, Kate Gatti
Approval date	02 February 2024

10. Appendix A: Notifications Matrix

Notifications matrix

The notifications matrix sets out the certain events in respect of a person whereby the mental health service must notify a third party.

The matrix also sets out:

- the person within the mental health service responsible for notifying the third party.
- the relevant section of the *Mental Health Act 2014*; and
- the approved form to be used in making the notification (if applicable)

Abbreviations

For the purposes of the notification's matrix:

- **MHAS** means the Mental Health Advocacy Service established as the Office of the Chief Mental Health Advocate under the *Mental Health Act 2014*.
- **MH Tribunal** means the Mental Health Tribunal established under the *Mental Health Act 2014*.
- **MIARB** means the Mentally Impaired Accused Review Board established under the *Criminal Law (Mentally Impaired Accused) Act 1996*.
- **OCP** means the Office of the Chief Psychiatrist established under the *Mental Health Act 2014*.

Event	Section of the MHA 2014	Person responsible	Party to be notified
Making a Form 3A – Detention order in respect of a referred person. (Refer to Form 3A)	s. 28(8)	The medical practitioner or AMHP who makes the order	PSP
Releasing a person because they cannot continue to be detained under a Form 3A - Detention order	s. 28(12)	The medical practitioner or AMHP who makes the order	PSP
Releasing a person who was being detained under a Form 3A – Detention order, because the referral was revoked (Refer to Form 3A)	s. 31(7)	The medical practitioner or AMHP who revokes the referral	PSP
Making a transport order to take a referred person to the place of examination (Refer to Form 4A)	s. 29(4)	The medical practitioner or AMHP who makes the order	PSP
Following an examination by a psychiatrist at an authorised hospital making either an: • inpatient treatment order in an authorised hospital, • a community treatment order, • an order continuing detention for further examination, or • an order that the person cannot continue to be detained (Refer to Form 6A, 5A, 3D or 3E)	s. 55(6)	The person in charge of the authorised hospital	PSP MHAS MH Tribunal MIARB*
Following an examination by a psychiatrist at a place that is not an authorised hospital making either an: • inpatient treatment order in general hospital, • a community treatment order, • an order authorising reception and detention at an authorised hospital for an examination by a psychiatrist, or • an order that the person cannot continue to be detained (Refer to Form 6B, 5A, 3D or 3E)	s. 61(5)	The psychiatrist who makes the order	PSP MHAS MH Tribunal MIARB*

Event	Section of the MHA 2014	Person responsible	Party to be notified
Transferring an involuntary inpatient from a general hospital to an authorised hospital (Refer to Form 4C)	s. 66(5)	The patient's treating psychiatrist	PSP MH Tribunal (with patient's consent)
In the case that an ITO needs to be confirmed - Releasing an involuntary inpatient from detention in an authorised hospital because: • the patient is on a 'Form 6B – Inpatient treatment order in a general hospital' and • the examination for this order took place via AV and • since that examination, the patient has not had a face-to-face examination and • the patient has since been transferred to the authorised hospital and • A 'Form 6D – Confirmation of inpatient treatment order' was not made by a psychiatrist at the authorised hospital with 24 hours of the patient being admitted to the authorised hospital.	s. 68(7)	The person in charge of the authorised hospital	PSP
Releasing an involuntary inpatient from detention in a hospital because, following an examination, the inpatient treatment order was revoked, or a community treatment order was made. (Refer to Form 5A, 6A or 6B)	s. 89(6)	The person in charge of the hospital	PSP
Releasing an involuntary inpatient from detention in a hospital because, at any time, the inpatient treatment order was revoked, or a community treatment order was made. (Refer to Form 5A, 6A or 6B)	s. 90(5)	The psychiatrist who makes the order	PSP MHAS MH Tribunal MIARB*

Transferring an involuntary inpatient between authorised hospitals (Refer to Form 4C)	s. 91(5)	The psychiatrist who makes the order	PSP MH Tribunal (with patient's consent)
When an inpatient treatment order expires.	s. 93(4)	The person in charge of the hospital at which the involuntary inpatient was detained	PSP MHAS MH Tribunal MIARB*
When a person is absent without leave from a hospital or other place.	s. 97(3)	The person in charge of the hospital or other place	PSP
Granting an involuntary inpatient leave of absence from the hospital. (Refer to Form 7A)	s. 105(13)	The psychiatrist who makes the order	PSP
Extending or varying an involuntary inpatient's leave. (Refer to Form 7B)	s. 106(4)	The psychiatrist who makes the order	PSP
Cancelling an involuntary inpatient's leave. (Refer to Form 7B)	s. 110(5)	The psychiatrist who makes the order	PSP
Following an examination of an involuntary community patient by a psychiatrist, either revoking the patient's community treatment order, or making an inpatient treatment order in respect of the patient. (Refer to Form 5A, 6A or 6B).	s. 120(7)	The supervising psychiatrist	PSP MHAS MH Tribunal MIARB*

At any time while a patient is on a community treatment order, either revoking the patient's community treatment order, or making an inpatient treatment order in respect of the patient. (Refer to Form 5A, 6A or 6B).	s. 123(8)	The supervising psychiatrist	PSP MHAS MH Tribunal MIARB*
In the case that an ITO needs to be confirmed - Releasing an involuntary inpatient from detention in an authorised hospital because: • the inpatient treatment order was in respect of the person whilst they were on a community treatment order and • the examination for making the order was conducted via AV and • since then, the patient has not had a face-to-face examination and • a 'Form 6D – Confirmation of Inpatient treatment order' was not made by another psychiatrist within 24 hours of the patient being admitted to the authorised hospital.	s. 124(7)	The person in charge of the authorised hospital	PSP MHAS MH Tribunal MIARB*
Making a transport order for an involuntary community patient who has failed to comply with a Form 5F - Order to attend. (Refer to Form 4A)	s. 129(5)	The medical practitioner or mental health practitioner who makes the order	PSP
Releasing an involuntary community patient after they have been detained (for up to 6 hours) at a place specified in a Form 5F - Order to attend.	s. 130(5)	The person in charge of the place specified in the order to attend	PSP
Making either an inpatient treatment order or revoking a community treatment order when an involuntary community patient has failed to comply with a Form 5F - Order to attend. (Refer to Form 5A, 6A or 6B)	s. 131(8)	The supervising psychiatrist	PSP MHAS MH Tribunal MIARB*

Providing urgent non-psychiatric treatment to an involuntary inpatient or to a mentally impaired accused detained in an authorised hospital. (Refer to Form 9B)	s. 242(5)	The person in charge of the authorised hospital	PSP OCP MIARB*
Making an order authorising the transfer of an involuntary inpatient from a WA hospital to an interstate mental health service. (Refer to Form 4D)	s. 555(4)	The person in charge of the hospital	PSP MH Tribunal (with patient's consent)
Approving the transfer of an involuntary inpatient from an interstate mental health service to a WA hospital. (Refer to Form 4E)	s. 557(6)	The person in charge of the hospital	PSP MH Tribunal (with patient's consent)

This document can be made available in alternative formats on request.

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